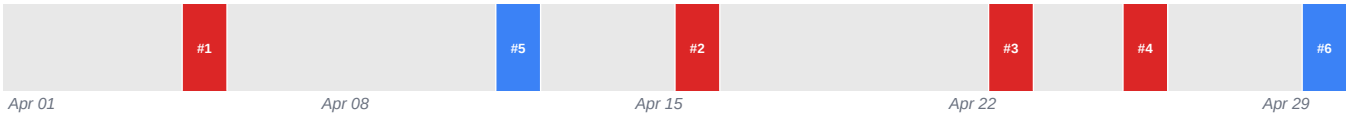


Patient ID: TaylorPeriod: April 1 to April 30, 2026Report Date: May 1, 2026Coverage: 658/720h (91%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 30 days from Apr 01 to Apr 30

■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
6	6h	Low Heart Rate	<1	91%

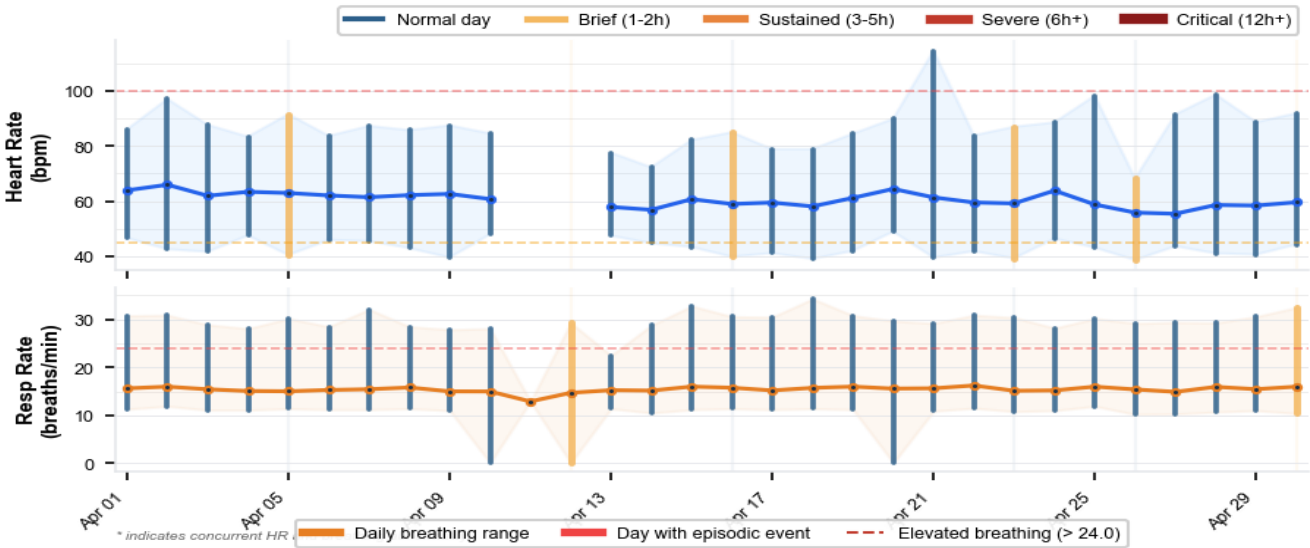
Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Low Heart Rate	39 bpm	1h	1h	1	42 bpm	Apr 05
2	Low Heart Rate	39 bpm	1h	1h	1	44 bpm	Apr 16
3	Low Heart Rate	39 bpm	1h	1h	1	43 bpm	Apr 23
4	Low Heart Rate	39 bpm	1h	1h	1	45 bpm	Apr 26
5	Elevated Breathing	29 brpm	1h	1h	1	29 brpm	Apr 12
6	Elevated Breathing	32 brpm	1h	1h	1	26 brpm	Apr 30

The P5 to P95 heart rate spread was 20 bpm (51 to 72), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

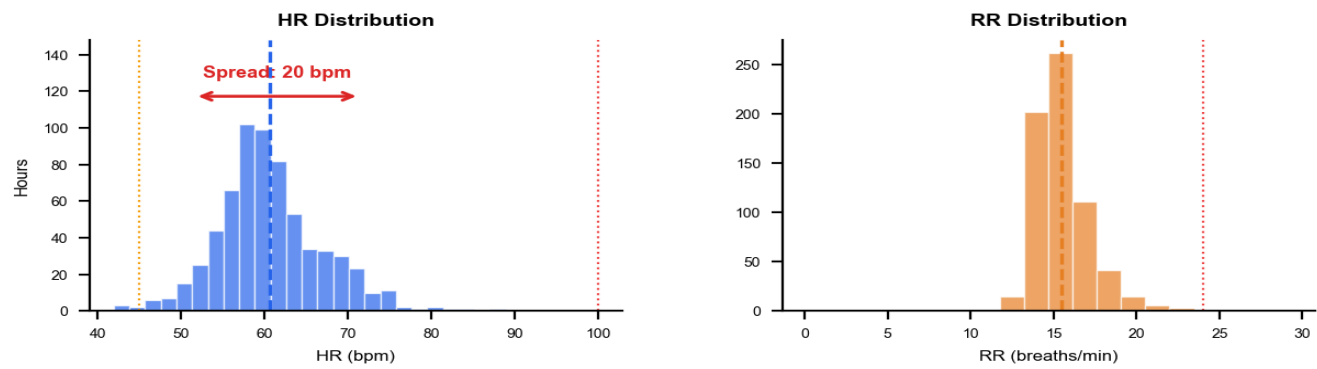
Clinical Pattern Observations:

1. High variability: HR spread 20 bpm (51 to 71).
2. Clustered pattern: Episodic events on 6 of 30 days (20%).

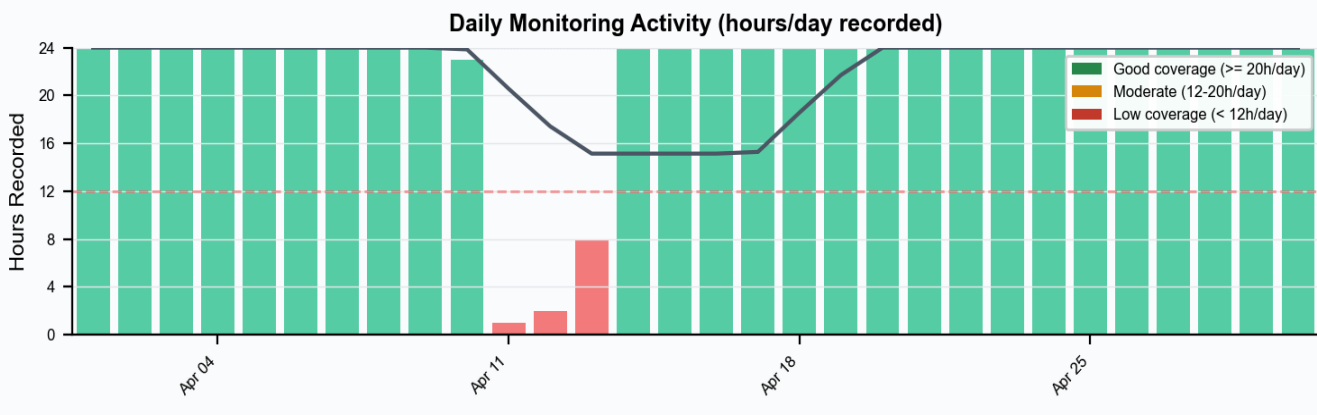


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

Very Low HR < 40 bpm Low HR < 45 bpm Elevated HR > 95 bpm High HR > 100 bpm
Very High HR > 110 bpm Elevated Breathing > 24 brpm High Breathing > 30 brpm Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.